Population Dynamics

CASE STUDY | Nigeria — Overpopulation

CASE STUDY | Australia — Underpopulation

CASE STUDY | Niger — High Population Growth Rate

CASE STUDY | Russia — Population Decline

CASE STUDY | China — Anti-natalist Policy (One-Child Policy)

CASE STUDY | Japan — Pro-natalist Policy

SYLLABUS OBJECTIVES | What you need to know

- Define birth rate, death rate, natural increase and population growth rate
- Explain the Demographic Transition Model (DTM) — all 5 stages
- Describe and explain causes of overpopulation and underpopulation
- Describe and explain causes of high and low population growth rates
- Explain the impacts of rapid population growth and population decline
- Evaluate anti-natalist policies (China One-Child Policy)
- Evaluate pro-natalist policies (Japan Angel Plan / Plus One Policy)
- Use population pyramids to analyse population structure

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1. Key Concepts and Definitions

Population dynamics is the study of how and why human populations change over time. The key measures below underpin every topic in this theme.

Term	Definition	Formula / Unit
Birth Rate (BR)	The number of live births per 1,000 people per year	Births / pop x 1,000 (per 1,000/yr)
Death Rate (DR)	The number of deaths per 1,000 people per year	Deaths / pop x 1,000 (per 1,000/yr)
Natural Increase	The difference between birth rate and death rate — does NOT include migration	NI = BR minus DR (per 1,000/yr)
Fertility Rate (TFR)	Average number of children a woman is expected to have in her lifetime	Replacement level = 2.1
Infant Mortality Rate	Number of deaths of children under 1 year old per 1,000 live births per year	per 1,000 live births/yr
Life Expectancy	The average number of years a person is expected to live from birth	Years
Overpopulation	Too many people relative to resources and technology to maintain an adequate standard of living	—
Underpopulation	Too few people to fully exploit a country's natural resources and maintain an adequate standard of living	—
Anti-natalist policy	Government policy designed to reduce birth rates and slow population growth	e.g. China One-Child Policy
Pro-natalist policy	Government policy designed to increase birth rates by encouraging larger families	e.g. Japan Angel Plan

2. The Demographic Transition Model (DTM)

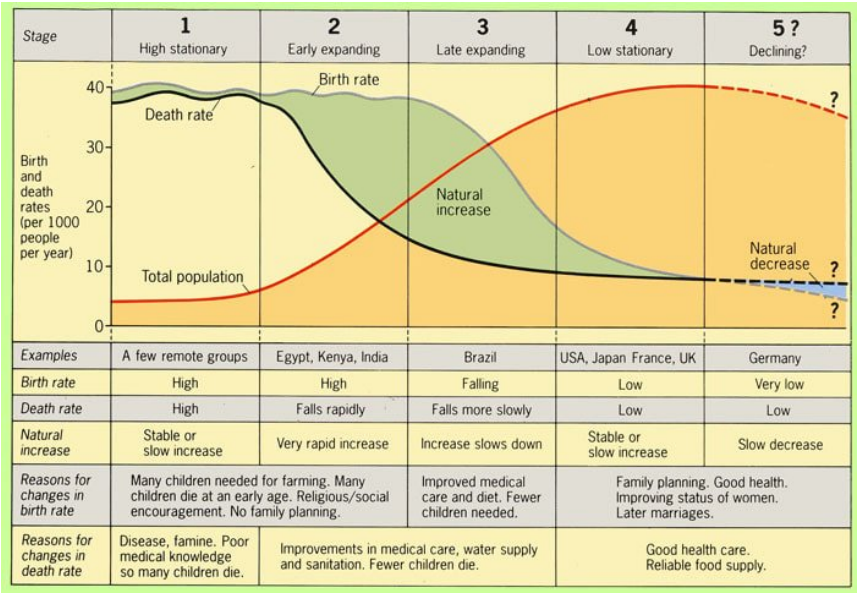

Fig 2.1 — The Demographic Transition Model showing birth rate (green), death rate (black) and total population (red) across 5 stages

Stage	Name	BR / DR	Population	Example	Key reason for change
1	High stationary	Both high	Stable, low	Remote groups	Disease, famine, poor medicine
2	Early expanding	BR high DR falling	Rapid increase	Niger, Egypt	Improved medicine, water, sanitation
3	Late expanding	BR falling DR low	Increase slows	Nigeria, Brazil	Family planning; improved female education
4	Low stationary	Both low	Stable, high	UK, USA, France	Good healthcare; reliable food; education
5	Declining	BR very low DR low	Slow decrease	Germany, Russia, Japan	Very low TFR; ageing society; career over family

EXAM TIP — DTM

Always link a country to a DTM stage WITH a reason.
e.g. 'Niger is in Stage 2 because its birth rate remains high at 46/1,000 while its death rate has fallen due to improved healthcare.'
The DTM does NOT account for migration — it only shows natural change (BR minus DR).

3. Overpopulation — Case Study: Nigeria

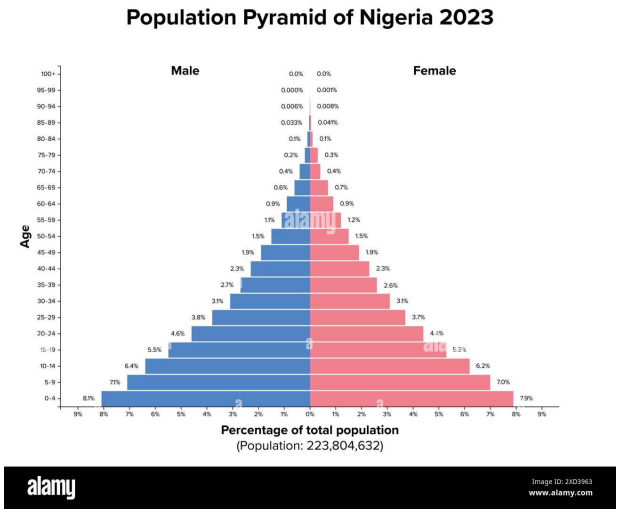

Fig 3.1 — Nigeria map (left) and population pyramid 2023 (right). The very wide base confirms a rapidly growing, youthful population.

FAST FACTS — NIGERIA

Population: 223.8 million (2023) | Growth rate: ~2.4% per year
Birth rate: ~35/1,000 | Death rate: ~11/1,000 | TFR: ~5.1 children per woman
DTM Stage: 3 (late expanding) | Life expectancy: ~54 years
66% of population under 25 | Lagos population: ~15 million (one of Africa's largest cities)

Causes of Overpopulation in Nigeria

• **High birth rate (TFR 5.1)**: Children are valued for economic security and farm labour; limited access to family planning, especially in rural north; early marriage is common.

• **Religious and cultural factors**: Large families are culturally celebrated; some religious groups oppose contraception, reducing uptake of family planning.

• **Falling death rate**: Improved healthcare reduces infant mortality — fell from ~120 to ~72 per 1,000 between 2000 and 2023 — while birth rate remains very high.

• **Rapid urbanisation**: Rural-to-urban migration concentrates people in cities, creating overcrowding in Lagos and Abuja even as rural areas remain less dense.

• **Population momentum**: 66% under age 25 — as this cohort reaches reproductive age, population will continue growing even if fertility starts to fall.

Problems Caused by Overpopulation in Nigeria

Problem	Detail
Housing shortage	Rapid urban growth creates slums — 60%+ of Lagos residents live in informal settlements such as Makoko
Overwhelmed services	Schools, hospitals and water systems cannot keep pace; only 1 doctor per 5,000 people nationally
Unemployment	Labour supply far exceeds demand; youth unemployment ~40%; most workers in low-paid informal sector
Food insecurity	Agricultural land cannot support population growth rate; Nigeria imports food despite having fertile farmland
Environmental pressure	Deforestation, water pollution and soil erosion increase as more land is cleared for farming and fuel

4. Underpopulation — Case Study: Australia

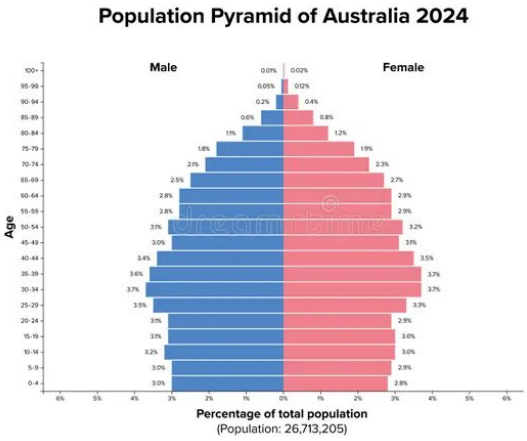

Fig 4.1 — Australia's vast, empty outback (left) and population pyramid 2024 (right). The even shape confirms a stable, developed population — but TFR is below replacement.

FAST FACTS — AUSTRALIA

Population: 26.7 million (2024) | Land area: 7.7 million km2 (3rd largest country)
Population density: ~3 people per km2 — one of the lowest in the world
Birth rate: ~13/1,000 | Death rate: ~7/1,000 | TFR: ~1.7 (below replacement of 2.1)
DTM Stage: 4 (low stationary) | Life expectancy: ~83 years | 86% urban (mainly coastal)

Causes of Underpopulation in Australia

• **Harsh physical environment**: ~70% of Australia is desert or semi-arid outback — extreme heat, very low rainfall and poor soils make large areas uninhabitable.

• **Low fertility rate (TFR 1.7)**: Below the replacement level of 2.1; high cost of living, career priorities and later marriages all reduce family size.

• **Vast land area**: Even with 26.7 million people, the land area creates extreme low density — the entire population could fit inside the city of Los Angeles.

• **Ageing population**: Growing elderly proportion reduces the working-age population and increases pressure on healthcare and pension systems.

• **Historical immigration limits**: The 'White Australia' immigration policy (abolished 1973) restricted population growth for decades.

Problems and Responses to Underpopulation

Problem	Australia's Response
Labour shortages in remote mining and agriculture	Active skilled migration programme — Australia recruits internationally to fill gaps
Underutilisation of vast natural resources	Foreign investment encouraged; mining sector depends heavily on migrant labour
Insufficient tax base to fund ageing population's services	Baby Bonus (A$5,000 per child, 2004-2013); ongoing family tax benefits
Remote communities lack healthcare, schools and infrastructure	'Fly-in fly-out' (FIFO) worker model; investment in regional development

5. High Population Growth Rate — Case Study: Niger

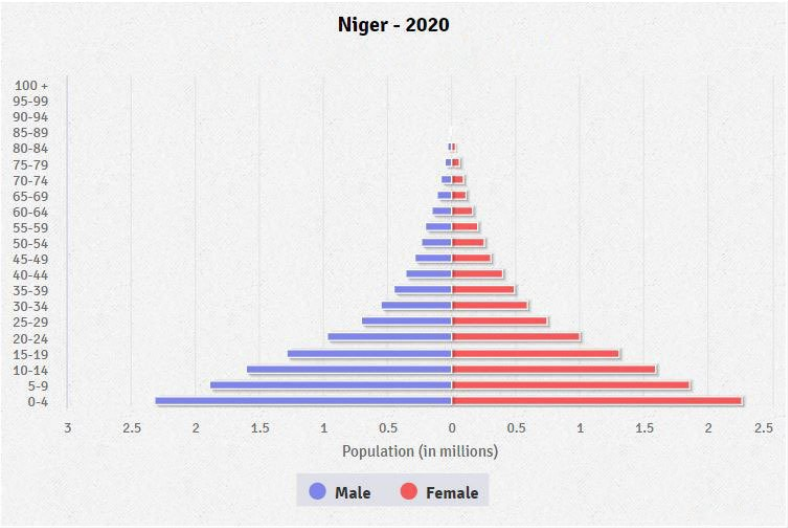

Fig 5.1 — Niger population pyramid 2020. The extremely wide base and very narrow top confirm one of the fastest-growing and youngest populations on Earth.

FAST FACTS — NIGER

Population: ~26 million (2024) | Growth rate: ~3.7% per year — one of the highest in the world
Birth rate: ~46/1,000 | Death rate: ~9/1,000 | TFR: 6.9 — HIGHEST IN THE WORLD
DTM Stage: 2 (early expanding) | Life expectancy: ~62 years
Infant mortality: ~64 per 1,000 live births | 75% of population under 30
One of the world's poorest countries: HDI rank 189 out of 191 (2023)

Reasons for Niger's Very High Growth Rate

• **World's highest TFR (6.9)**: Children provide economic security in old age and agricultural labour in Niger's predominantly rural, subsistence-farming society.

• **Early marriage**: Over 75% of girls marry before age 18, extending the reproductive period and directly increasing the number of children per woman.

• **Limited access to contraception**: Fewer than 12% of women use contraception — poor infrastructure, very low education levels and cultural barriers restrict access.

• **Falling death rate**: International aid, vaccination campaigns and improved basic healthcare have reduced the death rate from ~25 to ~9/1,000 since 1960 — but the birth rate has barely changed, creating massive natural increase.

• **Low female education**: Only ~11% of women complete secondary education. Research shows educated women consistently have fewer children.

• **Religious and cultural values**: Large families encouraged by Islamic tradition and seen as a source of status and security.

Consequences of Niger's Rapid Population Growth

Social	Economic	Environmental
Schools and hospitals overwhelmed; adult literacy rate only 35%; women unable to work due to repeated childbearing	Economy cannot absorb workforce; GDP per capita ~US$600; cycle of poverty deepens; food aid dependent	Overgrazing and deforestation as more land is cleared; desertification spreading in Sahel region
Very high child dependency ratio — each worker supports many dependants	Agricultural land degraded; Niger faces regular famine despite fertile river valley land	Water scarcity intensifying; soil exhaustion reducing crop yields further

6. Population Decline — Case Study: Russia

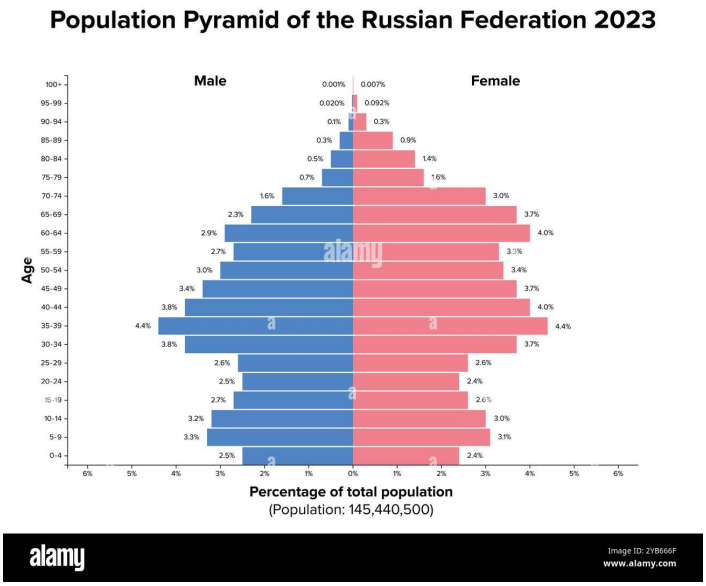

Fig 6.1 — Russia population pyramid 2023. The narrow base and wide middle/top confirms declining birth rate, an ageing population, and the striking male-female imbalance at older ages.

FAST FACTS — RUSSIA

Population: 145.4 million (2023) — declining since early 1990s
Birth rate: ~9/1,000 | Death rate: ~14/1,000 | Natural decrease: -5 per 1,000
TFR: ~1.5 — well below the replacement level of 2.1 | DTM Stage: 5 (declining)
Life expectancy: ~73 years overall | Men: ~68 years | Women: ~78 years
10-year gap between male and female life expectancy — one of the largest in the world

Causes of Population Decline in Russia

• **Low TFR (1.5)**: Career over family; high urban cost of living; limited affordable childcare; many couples choose to have one child or none.

• **Very high male death rate**: Russian men live on average only ~68 years — linked to very high rates of alcohol consumption, cardiovascular disease and accidents.

• **Post-Soviet economic collapse (1991)**: Economic uncertainty after the collapse of the USSR caused a dramatic birth rate crash in the 1990s — visible as a narrow cohort in the middle of the pyramid.

• **Emigration of young skilled workers**: Educated young Russians have moved to Western Europe and the USA, removing the most economically productive age group.

• **Impact of conflict (post-2022)**: Military casualties and mass emigration following the 2022 conflict have dramatically accelerated demographic decline.

Consequences and Russia's Pro-natalist Responses

Consequence	Russia's Response
Shrinking workforce supporting a growing elderly population	'Maternity Capital' scheme: cash payment (~750,000 roubles) for second child onwards
Pressure on pension and healthcare systems	Pension age raised (men: 65, women: 60) to keep more people in workforce longer
Military recruitment difficulties	Encouraged immigration from former Soviet republics (Kazakhstan, Uzbekistan)
Declining economic output as workforce shrinks	Extended maternity leave (up to 3 years); subsidised childcare expansion

7. Anti-natalist Policy — Case Study: China One-Child Policy

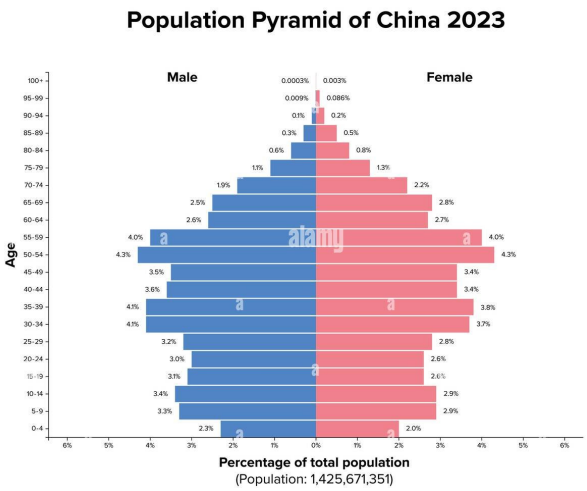

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Fig 7.1 — Chinese One-Child Policy propaganda poster (left): two parents, one child. The population pyramid (right) shows the narrow 20-35 age cohort born during the policy — and the ageing crisis now unfolding at the top.

FAST FACTS — CHINA ONE-CHILD POLICY

Policy introduced: 1979 | Ended: 2015 (two-child policy); 2021 (three-child policy)
Why introduced: population was 970 million and growing — feared food and resource crisis
Population 2023: 1.425 billion | TFR fell from ~5.8 (1960s) to ~1.6 (2023)
Birth rate: ~10/1,000 | DTM Stage: 4-5 | Exceptions: ethnic minorities, rural couples

How the Policy Worked — Incentives and Penalties

Incentives for compliance (one child)	Penalties for non-compliance (extra children)
Free education, healthcare and priority housing for single-child families Higher wages and better employment prospects in government jobs Financial bonuses for sterilisation after one child 'Certificate of Honour' awarded to single-child families	Fines of several years' income for additional children Demotion or dismissal from government employment Loss of free education and healthcare for any additional children Enforced sterilisation and abortion reported in some areas

Evaluation — Successes and Problems

Successes	Problems Created
TFR fell from ~5.8 to ~1.6 — dramatic slowing of growth GDP per capita rose sharply as fewer dependants Women gained more education and career opportunities Estimated 400 million births prevented Food and resource pressure reduced significantly	Gender imbalance: 120 boys born per 100 girls (sex-selective abortion) 'Little Emperor' syndrome — only children less resilient Rapidly ageing population now mirrors Japan's crisis Shrinking workforce creating economic problems Human rights abuses: forced sterilisation and abortion reported

8. Pro-natalist Policy — Case Study: Japan

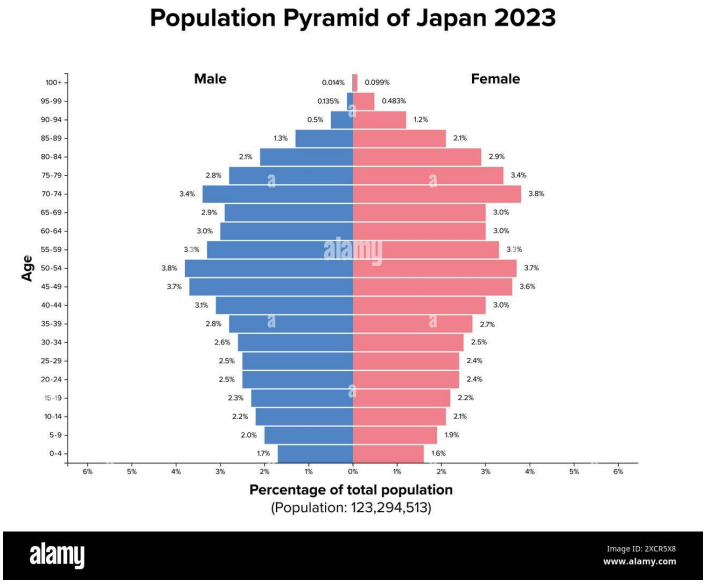

Fig 8.1 — Japan population pyramid 2023. The coffin shape — narrowest at the base, widest in the 45-74 range — shows a severely ageing, declining population.

FAST FACTS — JAPAN

Population: 123.3 million (2023) and falling — peaked at 128 million in 2010
Birth rate: ~7/1,000 | Death rate: ~13/1,000 | Natural decrease ongoing every year
TFR: ~1.2 (2023) — record low; one of the lowest in the world | DTM Stage: 5
Life expectancy: ~84 years (women: ~87, men: ~81) | 29% of population aged 65+
29% elderly — highest proportion of elderly citizens of any country in the world

Causes of Japan's Population Decline

• **Extremely low TFR (1.2)**: Highest-ranked economies have lowest fertility. High cost of living, expensive childcare and very long working hours deter couples.

• **Late and declining marriage rates**: Average age at first marriage is now 30+ for men; many young Japanese reject marriage entirely due to work pressure.

• **Cost and pressure of raising children**: Japan's demanding school culture, small urban apartments and extreme education costs make larger families very difficult.

• **Cultural pressure on women**: Women face an effective choice between career and family in Japan's long-hours workplace culture; many choose career.

• **Historically very low immigration**: Japan has resisted large-scale immigration unlike Australia or the UK, preventing the population boost immigrants provide.

Japan's Pro-natalist Policies — and Why They Have Largely Failed

Policy	Key Measures	Outcome
Angel Plan (1994)	Extended childcare; longer parental leave; reduced working hours for parents	Limited effect — TFR continued falling through 1990s
New Angel Plan (1999)	Enhanced childcare; financial incentives for 2nd+ children; work-life balance reforms	TFR stabilised around 1.3 briefly but did not rise
Plus One Policy (2002)	Encouraged each couple to have one more child than originally planned	Largely ineffective — root causes unaddressed
Child Allowance (2010-present)	Monthly payments (up to 15,000 yen/month per child); free pre-school from age 3 (since 2019)	Some stabilisation but TFR hit record low 1.2 in 2023
2023 Emergency Measures	Government declared 'demographic crisis'; tripled child benefit; major childcare expansion; immigration reforms debated	Too early to assess — TFR still falling

ANTI-NATALIST vs PRO-NATALIST — Key Comparison

China (anti-natalist): Compulsory policy — TFR fell dramatically from 5.8 to 1.6. EFFECTIVE at reducing growth BUT created gender imbalanc
Japan (pro-natalist): Voluntary incentives — TFR is STILL FALLING at 1.2 despite decades of policies. Root causes (culture and cost) are very
Exam point: Anti-natalist policies generally change TFR faster than pro-natalist policies, but often at significant social cost.

9. Reading and Analysing Population Pyramids

A population pyramid shows the age-sex structure of a population. Each horizontal bar = one 5-year age group. Shape reveals DTM stage and trend.

Shape	Type	What it tells you	Examples
Very wide base, narrow top	Expansive (Stage 2)	High BR and DR; low life expectancy; rapid growth; very high child dependency ratio	Niger, Stage 2 Nigeria, Stage 3
Tapering sides, some width at base	Transitional (Stage 3)	Falling BR; falling DR; increasing life expectancy; growth slowing; more even spread of ages	Brazil, India
Roughly even bars, slight taper	Stationary (Stage 4)	Low BR and DR; high life expectancy; stable population; balanced age structure; low dependency ratio	Australia, USA, UK
Narrowest base, bulge in middle/top	Declining (Stage 5)	Very low BR; low DR; very high life expectancy; shrinking population; very high old-age dependency	Japan, Germany, Russia

Dependency Ratio

The **dependency ratio** measures the proportion of non-working (dependent) people relative to the working-age population.

DEPENDENCY RATIO FORMULA

Dependency ratio = (population 0-14 + population 65+) / population 15-64 x 100
A high ratio means more dependants per worker — a greater economic burden on the working population.
A ratio above 50 means more than 50 dependants per 100 workers.
Niger: very HIGH (dominated by young dependants). Japan: very HIGH (dominated by elderly dependants).
Both types of high dependency create economic strain — but for very different reasons.

10. Key Vocabulary — 20 Terms You Must Know

Term	Definition
Birth rate	Number of live births per 1,000 people per year.
Death rate	Number of deaths per 1,000 people per year.
Natural increase	Birth rate minus death rate — does NOT include migration.
Fertility rate (TFR)	Average number of children a woman has in her lifetime. Replacement = 2.1.
Infant mortality rate	Deaths of children under 1 year old per 1,000 live births per year.
Life expectancy	Average number of years a person is expected to live from birth.
Population pyramid	Bar chart showing the age and sex structure of a population.
Dependency ratio	Ratio of non-working (0-14 and 65+) to working-age (15-64) population x 100.
DTM	Demographic Transition Model — 5-stage model showing how BR and DR change with development.
Overpopulation	Too many people relative to resources — standard of living falls.
Underpopulation	Too few people to fully exploit a country's resources and potential.
Anti-natalist policy	Government policy to reduce birth rates (e.g. China One-Child Policy 1979-2015).
Pro-natalist policy	Government policy to increase birth rates (e.g. Japan Angel Plan 1994).
Population momentum	Continued growth even after fertility falls, due to large numbers of young people.
Youthful population	Population with high proportion of young people — typical of DTM Stage 2 or 3.
Ageing population	Population with high proportion of elderly — typical of DTM Stage 4 or 5.
Gender imbalance	Unequal sex ratio — e.g. China's surplus of males following the One-Child Policy.
Maternity Capital	Russia's cash payment to encourage families to have a second child.
Natural decrease	When death rate exceeds birth rate — total population shrinks without migration.
Population momentum	Continued growth even if TFR falls, because large young cohorts still reproduce.

11. Syllabus Checklist — Cambridge 0460 Theme 1.1

- ■ Define birth rate, death rate, natural increase, TFR and infant mortality rate
- ■ Describe and explain all 5 stages of the Demographic Transition Model (DTM)
- ■ Link named countries to appropriate DTM stages with supporting reasons
- ■ Define and distinguish between overpopulation and underpopulation
- ■ Describe causes and problems of overpopulation — Nigeria case study with data
- ■ Describe causes and problems of underpopulation — Australia case study with data
- ■ Describe causes and consequences of high population growth rate — Niger (TFR 6.9)
- ■ Describe causes and consequences of population decline — Russia (TFR 1.5, Stage 5)
- ■ Describe and evaluate an anti-natalist policy — China One-Child Policy (1979-2015)
- ■ Describe and evaluate a pro-natalist policy — Japan Angel Plan / child allowance
- ■ Read and describe population pyramids — identify expansive, stationary and declining shapes
- ■ Calculate and interpret the dependency ratio using the formula

- Explain population momentum and why populations keep growing after fertility falls
- Compare the effectiveness of anti-natalist vs pro-natalist policies with named examples
- Use specific data and statistics in every exam answer

12. Exam Alert — Common Mistakes and Mark-Winning Tips

COMMON MISTAKES TO AVOID

Do NOT confuse Niger (Stage 2, TFR 6.9) and Nigeria (Stage 3, TFR 5.1) — different case studies.
Do NOT say natural increase includes migration — it is ONLY birth rate minus death rate.
Do NOT describe a pyramid shape without saying what it means about BR, DR and development.
Do NOT say China's policy just 'worked' — you must evaluate: successes AND problems.
Do NOT say Japan's pro-natalist policies 'worked' — TFR is still falling at a record low 1.2.
Do NOT mix up anti-natalist (reduce growth) and pro-natalist (increase growth).
Do NOT forget to use data — vague answers lose marks; specific statistics gain them.

MARK-WINNING TIPS

Use specific data in every answer: 'Niger's TFR is 6.9 — the world's highest'; 'Japan's TFR is 1.2 — a record low'; 'China's policy reduced TF
For pyramid descriptions: comment on shape, base width, top width AND link each observation to a cause (e.g. 'the narrow base suggests a fa
For 'explain' questions: use connectives — 'because', 'therefore', 'as a result', 'this means'.
For 4-mark questions: give 2 fully developed points with explanation, not 4 thin points.
For 7-mark questions: use PEEL — describe, explain, evaluate with named data and a conclusion.
Always name your case study: 'In Nigeria...'; 'China's One-Child Policy...' — never 'a country'.

13. Exam Practice Questions — Cambridge 0460 Style

All questions follow Cambridge 0460 Paper 1 style. Section B uses the 1 + 2 + 3 + 4 + 7 mark structure.

Question 1

[1 marks]

(a) What is the term for the average number of children a woman is expected to have during her lifetime? [1]

MARK SCHEME

1 mark: Total Fertility Rate (TFR) ✓ (accept: fertility rate)

Question 2

[2 marks]

(b) State two reasons why Niger has one of the highest population growth rates in the world. [2]

MARK SCHEME

1 mark each for any two correct, distinct reasons (max 2):

World's highest TFR of 6.9 ✓ | Early marriage — 75%+ of girls marry before 18 ✓

Very low contraceptive use (under 12%) ✓ | Children valued for farm labour and old-age security ✓

Falling death rate due to improved healthcare ✓ | Very low female education levels ✓

Question 3

[3 marks]

(c) Explain the differences between an anti-natalist policy and a pro-natalist policy. [3]

MARK SCHEME

1 mark per clear, correct, developed point (max 3):

Anti-natalist: designed to REDUCE birth rate / slow population growth ✓

uses penalties/disincentives to discourage large families ✓; e.g. China OCP fines ✓

Pro-natalist: designed to INCREASE birth rate / reverse population decline ✓

uses financial incentives/payments to encourage more children ✓; e.g. Japan child allowance ✓

Question 4

[4 marks]

(d) For a named country you have studied, describe and explain the problems caused by a rapid population growth rate. [4]

Named country: _____

MARK SCHEME

Credit well-developed problems with explanation and named data (max 4 marks):

Nigeria: housing shortage — 60%+ in Lagos slums ✓ + explanation ✓

Overwhelmed services — 1 doctor per 5,000 people ✓ + explanation ✓

Unemployment — youth unemployment ~40% ✓ + explanation ✓

Food insecurity, environmental pressure — credit with development ✓

Must name a country. Max 2 without named data or explanation.

Question 5 — Extended Writing [7 marks]

[7 marks]

(e) For a named country you have studied, describe and evaluate the effectiveness of a population policy. [7]

Named country and policy: _____

MARK SCHEME

Level 3 [6-7]: Policy described in detail with specific data; balanced evaluation of both successes AND failures; supported conclusion reached.
e.g. China OCP: TFR fell 5.8 to 1.6 (success) BUT gender imbalance 120:100, ageing population, human rights abuses (problems). Policy reversed 2021.

Level 2 [4-5]: Policy described with some evaluation; limited data; mainly one-sided.

Level 1 [1-3]: Basic description only; policy named but little detail or evaluation.

Accept Japan: Angel Plan described; evaluate — TFR still 1.2, policies largely failed.

PEEL FRAMEWORK — use for ALL 7-mark extended answers

P	**POINT**	Open with a clear geographical statement that directly answers the question.
E	**EVIDENCE**	Back it up with specific data — country names, statistics, dates, policies.
E	**EXPLAIN**	Explain the geographical reason WHY — use 'because', 'therefore', 'this means'.
L	**LINK**	Link back to the question wording. Develop or add a contrasting example.

14. Summary Card — Cut Out and Revise

CASE STUDY	KEY FACTS	MAIN CAUSES	KEY OUTCOMES / POLICY

Nigeria Overpopulation Stage 3	Pop: 224m TFR: 5.1 BR: 35, DR: 11 Growth: 2.4%/yr	High TFR; falling DR; cultural value of children; early marriage; no family planning	Housing shortage (Lagos slums); overwhelmed services; ~40% youth unemployment
Australia Underpopulation Stage 4	Pop: 26.7m Density: 3/km2 TFR: 1.7 LE: 83 yrs	Desert interior (70%); low TFR; vast land area; historical immigration limits	Labour shortages; underused resources; Baby Bonus policy; skilled migration
Niger High growth Stage 2	Pop: 26m TFR: 6.9 Growth: 3.7%/yr HDI: 189/191	TFR 6.9 (world's highest); early marriage; no contraception; low female education	Poverty cycle; overwhelmed services; desertification; food insecurity
Russia Decline Stage 5	Pop: 145m (falling) TFR: 1.5 BR: 9, DR: 14 LE: 73 yrs	Low TFR; high male mortality (alcohol); 1991 economic crash; emigration	Shrinking workforce; pension pressure; Maternity Capital pro-natalist response
China Anti-natalist Stage 4-5	OCP: 1979-2015 TFR: 5.8 to 1.6 Pop: 1.425bn BR: 10/1,000	One-Child Policy: fines for extra children; incentives for compliance	SUCCESS: 400m births prevented. PROBLEMS: gender imbalance; ageing; human rights
Japan Pro-natalist Stage 5	Pop: 123m (falling) TFR: 1.2 (record low) 29% aged 65+ LE: 84 yrs	Angel Plan (1994); child allowances; parental leave; free pre-school	LARGELY FAILED — TFR still falling. Root causes (cost, culture) too deep to fix with money

EXAM REMINDER

Cambridge 0460 Paper 1 Section B: 1 + 2 + 3 + 4 + 7 mark structure.
Always name your case study: 'In Nigeria...'; 'China's One-Child Policy...' — never just 'a country'.
7-mark answer needs: description + explanation + named data + evaluation + conclusion.
Key comparisons to revise: Niger (Stage 2) vs Japan (Stage 5) | China (anti) vs Japan (pro).